

AllHealth CHOICE

Population Health Management Report

Time Period:

Medical Data: 2022-01-01 To 2024-04-01

Pharmacy Data: 2022-07-01 To 2024-03-09

Prepared for

Long County

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1. Executive Summary

Introduction:

The following report is the result of an analysis of archival medical and pharmacy utilization data. The intent of this analysis is to yield a better understanding of the epidemiology currently influencing this population and to suggest population health management opportunities that can address the specific risk impacting this population. In order to accomplish this task, archival data was processed through proprietary algorithms in order to properly risk-stratify the population. The risk of a population has a direct relationship to current and future spending patterns. Variables that are the building blocks of risk and/or disease include, but are not limited to:

- Age, Gender, Lifestyle, Genetics, Chronic Illness, Co-Morbidities, Multi-Morbidities, Medication Compliance/Non-Compliance, Compliance/Non-Compliance to Evidence-Based Guidelines, Gaps in Care, etc.

The majority of the aforementioned variables were utilized to investigate risk stratifications within the population. The overall health of a population is determined by multiple factors; however, an individual's lifestyle is a powerful predictor of leading causes of morbidity and disability.

This analysis explored multiple areas of interest within the data, including the following research questions:

1. What is the cost burden of lifestyle modifiable risk factors within the employee population?
2. What is the relationship of age and gender to various disease states?
3. What are the gaps in care associated with suggested preventive measures for this population?
4. What is the relationship between drug compliance and non-compliance, as related to disease severity?
5. What is the financial burden associated with chronic disease within this population?
6. What is the level of HEDIS compliance (i.e., evidence-based & preventive medicine) within this population?
7. What is the expense related to specific co-morbidities (i.e., hypertension, hyperlipidemia, depression, etc.) within this population?
8. What variables best predict and explain future high spenders within this population?
9. What are actionable solutions that can be implemented to mitigate existing and future health risks?

Key Findings and Solutions for Consideration:

The following key findings resulted from the analysis of archival health care data.

Key Finding 1: Chronic Disease

Key Finding: Patterns of risk generally occur within any given population. In order to better understand these patterns, the population was risk stratified into five distinct groups:

Group	Description
Disease Group-1	No chronic disease and less than \$1,500 medical expenditures per 12 months
Disease Group-2	No chronic disease and \$1,500 or more medical expenditures per 12 months
Disease Group-3	One Chronic Disease
Disease Group-4	Two Chronic Disease
Disease Group-5	Three Chronic Disease
Disease Group-6	Four Chronic Disease
Disease Group-7	Five or More Chronic Disease

the number of individuals incorporated into each analysis.

Risk Group Summary Risk Group	2022			2023			2024		
	Risk Group Summary Total \$	Risk Group Summary Mean \$	Risk Group Summary N	Risk Group Summary Total \$	Risk Group Summary Mean \$	Risk Group Summary N	Risk Group Summary Total \$	Risk Group Summary Mean \$	Risk Group Summary N
GROUP-1	\$19,959	\$407	49	\$30,245	\$432	70	\$15,686	\$285	55
GROUP-2	\$87,538	\$5,471	16	\$180,545	\$7,523	24	\$19,126	\$2,732	7